E-Learning Program

Employee Induction E-Learning Program

- Six interactive SCORM courses: HSE · HR · Operations & My Care · RCM · Quality · IT (EHR)

Presented To

Human Capital Department
Learning & Development

My Clinic (عيادتي)
Jeddah, Kingdom of Saudi Arabia

Proposal date: 14-10-2026
Proposal Ref# e2026-MC01 Rev.01

Revision Sheet

REVISION NUMBER	DATE OF REVISION	REVISION DESCRIPTION
00	14-10-2026	Initial proposal
01	14-10-2026	Narration updated to ~60 min from the finished scripts; RCM scenario count 4 (total 26); two guide characters (Noor and Dr. Omar) and two narration voices added; full draft storyboards now prepared

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About Leap to Success™

Leap To Success™ is a consulting, learning, and development company established in 2012 located in Dubai, United Arab Emirates delivering its services all over the Gulf region. The name of our company describes how we work with our customers, we work collaboratively with our customers to help them make the LEAP towards success and growth.

Unlike the conventional way, we deliver our services by considering customers' tailored needs and specific long-term strategic goals in the delivery process, and not only the requirements of a job description.

Leap To Success™ focuses on providing high-quality learning services by ensuring that our experts carry comprehensive consultations with our customers in order to guarantee that the training and content meet the requirements.

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Executive Summary

MY CLINIC (عيادتي) currently delivers its new-employee induction as a full-day classroom programme supported by slide decks, PDF manuals, a speaker script and two recorded videos. **MY CLINIC** has asked **L2S** to turn this material into interactive e-learning courses that every new employee can complete online, at their own pace, with progress and results tracked in the Learning Management System (LMS).

Before writing this proposal, **L2S** reviewed every file in **MY CLINIC**'s induction folder: two Health & Safety manuals (96 pages), three PowerPoint decks (82 slides), one Word speaker script and two videos (17.5 minutes, reviewed through more than 200 screenshots). For each course we have already prepared a phased production plan with learning objectives, a full screen-by-screen draft storyboard (all 207 screens), a narration script, an asset list, build settings and test cases. This proposal is based on that analysis, so the scope, effort and price below reflect the real content, not an estimate.

L2S proposes to design, develop, test and deliver **six bespoke induction courses**:

- **Health, Safety & Environment (HSE)** — ~90 min of learner time
- **Welcome On Board: HR Essentials** — ~45 min of learner time
- **Who Does What: Operations & My Care** — ~15 min of learner time
- **RCM: Documentation, Coding & EBP Benefits** — ~45 min of learner time
- **Clinical Governance: Quality, Patient Safety & Risk** — ~25 min of learner time
- **IT: EHR Consultation Workflow** — ~25 min of learner time

Together, the six courses contain about **207 interactive screens**, **95 scored activities**, **26 scenarios, games and guided simulations**, about **60 minutes of professional narration in two voices** and close to **4 hours of learner seat time**. Each course is priced at **USD 1,500**, for a total investment of **USD 9,000**.

Needs

L2S understands the needs of **MY CLINIC** to be:

- Replace a full day of classroom induction with self-paced online courses that new staff can complete in their first week, at any branch and from any device.
- Give every branch the same, accurate message on safety, HR policy, patient safety, documentation and systems.
- Prove competence, not just attendance: scored practice, pass marks and per-objective results that support CBAHI survey readiness.
- Turn passive slides and videos into realistic practice: fire and spill emergencies, patient identification, leave requests, claim documentation and the EHR consultation workflow.
- Keep sensitive data out of training: no patient identifiers, anonymised claim data and role-based (not name-based) organisation charts.
- Make the courses easy to update when policies, codes or systems change.

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Our Approach: What L2S Will Do

L2S will act as the instructional design and development partner for the whole programme. We will lead every phase, from source analysis to LMS launch, and work with **MY CLINIC**'s subject-matter experts (SMEs) at defined review points. Our approach is built on the following principles:

- **Action Mapping.** We start from what staff must *do* on the job (announce the right emergency code, identify a patient with two identifiers, code a specific diagnosis, complete a consultation in the EHR) and build every activity around those actions. Content that does not support an action becomes a reference job aid, not a slide.
- **Scenario-based, hands-on practice.** Learners make decisions and see the consequences: a Code Red drill, a spill triage, a "leave desk" of colleagues' requests, a claim to rework, a step-by-step EHR simulation on a test patient.
- **One idea per screen.** Clean, minimalist screens in the MY CLINIC brand. Key words on screen; the explanation is in the narration.
- **Professional narration with captions.** Every narrated screen has captions and a transcript.
- **Two guide characters.** Noor, a senior nurse, and Dr. Omar, a family physician, guide learners through every course: they welcome, give tips, host the scenarios and close each module, with one consistent voice each across the whole programme.
- **Accessibility.** Designed to WCAG 2.2 AA: keyboard operable, alt text on every meaningful image, colour never the only cue (important for emergency codes, NFPA diamonds and risk matrices).
- **Standards and tracking.** SCORM 1.2 or SCORM 2004 packages with completion, score, objective-level and question-level reporting, tested in SCORM Cloud and in MY CLINIC's LMS.
- **Quality gates at every stage.** Automated validation of storyboards, narration and packages, plus SME approval before any media is produced.
- **Privacy by design.** No real patient data, staff data or claim numbers. EHR and Oracle screens are re-captured on test accounts.

Work Already Completed (Discovery)

The following source analysis is complete and is included in this proposal at no extra cost to **MY CLINIC**:

Source file	Size	What L2S extracted
Health & Safety 1.pdf	65 pages, 9 modules	Full text and every page reviewed; 9 FMS plans, 11 emergency codes, RACE/PASS, hazmat, radiation and laser content mapped to activities
Health & Safety 2.pdf	31 pages, mostly visual	Every page reviewed; hazard picture, bilingual code poster, evacuation tools, spill kits and chemical register turned into practice activities
HR.pptx	62 slides	All text and images; values, 7C strategy, ethics, patients' rights, leave, GOSI, overtime, Oracle, Qiwa and Mumaris content

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Source file	Size	What L2S extracted
Operations & My Care Structure.pptx	6 slides	Activities and org charts; plan to teach roles and routing, not names
RCM script (.docx)	16 sections	Documentation, ICD-10, defensive medicine and rejection-cause content
RCM 2026 deck (.pptx)	14 slides	New EBP added and amended benefits; rejection log (to be anonymised)
Quality, Safety & Risk video	8 min 56 s, narrated	107 screenshots, 10-second narration log; five-part content rebuilt as interactive screens
IT video	8 min 38 s, silent	104 screenshots, on-screen text log; workflow mapped into 7 guided simulations

During this review L2S also logged **80 course-level questions** and **14 programme-level decisions** for MY CLINIC's SMEs, for example the Code Orange spill threshold (the two Health & Safety files disagree), the real emergency extensions (the slides show a placeholder), branch names, the regulator's name (CCHI vs CHI) and several HR policy figures. These are resolved in Phase 0 so that every fact in the courses is correct.

L2S also found that about one minute of the IT video shows a real patient's record and contact details. **That video will not be used.** The EHR course will be built on new screenshots of a test patient, and L2S recommends MY CLINIC restricts sharing of the original file.

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Course Catalogue

Summary of the six courses (all figures are planned, based on the completed source analysis):

#	Course	Built from	Audience	Seat time	Screens	Scored activities	Price (USD)
1	Health, Safety & Environment (HSE)	H&S 1 (65 pp) + H&S 2 (31 pp)	All new staff	~90 min	80	37	1,500
2	Welcome On Board: HR Essentials	HR deck (62 slides)	All new employees	~45 min	36	15	1,500
3	Who Does What: Operations & My Care	Operations deck (6 slides)	Front-line, PX & clinical staff	~15 min	13	4	1,500
4	RCM: Documentation, Coding & EBP Benefits	RCM script + RCM 2026 deck (14 slides)	Physicians, dentists, RCM staff	~45 min	36	17	1,500
5	Clinical Governance: Quality, Patient Safety & Risk	Quality video (8 min 56 s)	All clinical staff	~25 min	22	11	1,500
6	IT: EHR Consultation Workflow	IT video (8 min 38 s, silent)	Physicians & clinical staff	~25 min	20	11	1,500
	Total	**8 source files**		**~4 hours**	**207**	**95**	**9,000**

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Course 1 — Health, Safety & Environment (HSE)

Item	Details
Format	Two SCORM packages: Part 1 "Facility Management & Safety" (knowledge, ~55 min) and Part 2 "Safety in Practice Lab" (hands-on practice, ~35 min)
Audience	All new staff, clinical and non-clinical, in their first week
Source	Health & Safety 1.pdf (65 pages, aligned to CBAHI, MOH codes and ISO 45001) and Health & Safety 2.pdf (31 pages)
Pass mark	80%, completion = all screens viewed and pass mark reached
Price	USD 1,500

Learning objectives — on completion, learners will be able to:

- Name the eight CBAHI Facility Management & Safety plans and what each one protects.
- Apply the AREC cycle and the hierarchy of controls to a workplace hazard, and estimate risk as likelihood × impact.
- Match each MOH emergency code to its meaning and follow the common response pattern.
- Respond to a fire using RACE and PASS, choosing the correct extinguisher, evacuation type and evacuation tool.
- Read GHS pictograms, the NFPA diamond and the department chemical register; decide on Code Orange; use the spill kit; segregate waste.
- Apply security, utility and biomedical equipment rules (ID, visitors, emergency sockets, out-of-service tagging).
- Stay safe near radiation and lasers using ALARA, dose-limit bands and laser classes.

Course structure:

- Part 1: Welcome · The eight FMS plans · Occupational Health & Safety · Emergency codes · Fire safety · Hazardous materials & waste · Security · Utilities · Medical equipment · Radiation & laser · Capstone & completion checklist
- Part 2: Spot the hazard · Code Red drill · Evacuation · Extinguishers · Spills · Labels & chemical register · Hospital regulations

Key interactive activities:

- Explore the eight plans, the fire equipment in a clinic corridor and a treatment room's utilities (clickable illustrations).
- Sort RACE, PASS, the hierarchy of controls and the spill-kit procedure into the right order.
- Decision scenarios: a trailing cable in a waiting area, a night-shift fire, three spills, an overdue infusion pump.
- "Code Red drill" branching game: rescue, the alarm call script, contain, extinguish or evacuate, evacuation chair, assembly point.
- Store-room hazard hunt with six hidden hazards, then a risk-scoring exercise.

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- Drag-and-drop: extinguisher classes, evacuation tools, waste bins, allowed vs prohibited items.
- “Chemical store lockout” escape room using the NFPA diamond, the chemical register, eyewash time and the Code Orange rule.
- Capstone game “One shift, three plans” that combines fire, hazmat and biomedical decisions.

Assessment and tracking:

37 scored activities feed a results breakdown by objective, so each learner and the Safety & Security Manager can see which plan needs another look. Question-level results are reported to the LMS.

Course-specific deliverables:

- 2 SCORM packages
- Bilingual (English/Arabic) emergency-code flashcards
- Completion checklist (evacuation route, assembly point, nearest eyewash, safety liaison, acknowledgement)

Production volume:

≈80 screens · 37 scored activities · 8 scenarios, games and escape rooms · ≈26 diagrams and illustrations (fire triangle, NFPA diamond, GHS set, floor plan, ALARA, scenario scenes) · ≈23 minutes of narration · 26 course-specific test cases.

Inputs needed from MY CLINIC:

- Confirmed Code Orange threshold and the facility's emergency-code sheet with real extensions
- Waste-bin colours, emergency-socket colour and the Radiation Safety Officer's confirmation of dose bands
- Photos of an eyewash station, spill kits and an extinguisher label; one anonymised chemical register
- Safety & Security Manager, Biomedical Engineering and RSO reviews

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Course 2 — Welcome On Board: HR Essentials

Item	Details
Format	One SCORM package (~45 min)
Audience	All new employees, day 1–3; first course in the induction path
Source	HR.pptx (62 slides): company story, values, strategy, ethics, patients' rights and HR policies
Pass mark	70%
Price	USD 1,500

Learning objectives — on completion, learners will be able to:

- Link MY CLINIC's purpose, four values and 7C strategy to everyday actions in their role.
- Apply the code of ethics and patients' rights and responsibilities to workplace situations.
- Decide whether a leave request is eligible and how many days apply (annual, sick, exam, scientific, Hajj, compassionate, maternity, work injury).
- Calculate a GOSI deduction and an overtime payment.
- Use the right system for each HR task (Oracle, intranet, Qiwa, Mumaris) and submit a leave request in Oracle.
- Apply ID card, dress-code, contract and resignation rules.

Course structure:

- Welcome · Values & strategy · Our clinic (history, branches, leadership, awards) · Ethics & patients' rights · Your leave · Pay & social insurance · Systems & licences · ID, benefits & dress code · Contracts & leaving · Close

Key interactive activities:

- Icebreaker poll and values-in-action matching.
- Ethics decision scenario: a conflict of interest in a supplier evaluation.
- Sort patient rights vs patient responsibilities.
- “The leave desk”: handle four colleagues' leave requests correctly.
- Adaptive practice on leave rules — it gives extra questions only to learners who need them.
- Worked examples then practice for GOSI (e.g. 585 SAR) and overtime calculations.
- Guided Oracle simulation: request annual leave step by step (test account).
- Task-to-system matching and dress-code sorting by role.

Assessment and tracking:

15 scored activities with results by objective. Detailed tables (leave entitlements, GOSI, overtime) move into a downloadable HR Quick Reference, so the course focuses on applying the rules.

Course-specific deliverables:

- 1 SCORM package
- HR Quick Reference PDF (2 pages)

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- Link to the onboarding satisfaction survey

Production volume:

≈36 screens · 15 scored activities · 4 scenarios, simulations and adaptive practice · ≈11 visual sets (values and 7C icons, branch map, dress-code illustrations, Oracle screenshots) · ≈12 minutes of narration · 12 course-specific test cases.

Inputs needed from MY CLINIC:

- Confirmation of HR policy figures (sick-leave pay bands, compassionate leave, overtime eligibility, ID penalty, GOSI rates)
- Official branch names and codes; consent for any leadership photos
- An Oracle test account for screenshots
- Head of Human Capital review

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Course 3 — Who Does What: Operations & My Care

Item	Details
Format	One SCORM package (~15 min)
Audience	Front-line, patient-experience and clinical staff
Source	Operations & My Care Structure.pptx (6 slides)
Pass mark	70%
Price	USD 1,500

Learning objectives — on completion, learners will be able to:

- Distinguish the activities owned by Operations from those owned by My Care.
- Route common patient requests to the right team and role, first time.
- Explain the main roles and abbreviations (PXO, coordinators, DMP, CHP, HHC and others).

Course structure:

- The patient journey · Operations · My Care · Practice: right team, first time

Key interactive activities:

- Clickable patient journey showing which team leads at each stage.
- Role-based organisation charts, redrawn without names or headcounts (names stay in the live directory).
- Sort 10 activity cards between Operations and My Care.
- "Who do I call?" scenario with four real-style patient requests.

Assessment and tracking:

4 scored activities with results by objective.

Course-specific deliverables:

- 1 SCORM package
- Two redrawn role-based organisation charts

Production volume:

≈13 screens · 4 scored activities · 1 routing scenario · ≈6 diagrams and scenes · ≈4 minutes of narration · 8 course-specific test cases.

Inputs needed from MY CLINIC:

- Confirmed reporting lines and abbreviations
- Routing rules for the scenario
- Director of Operations and My Care Manager review

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Course 4 — RCM: Documentation, Coding & EBP Benefits

Item	Details
Format	Two SCORM packages: Part A "Documentation & ICD-10 Coding That Gets Approved" (~25 min) and Part B "New EBP Benefits & Claim-Rejection Analysis" (~20 min)
Audience	Physicians and dentists; RCM and insurance-approval staff (Part B)
Source	RCM speaker script (.docx) and RCM 2026 slide deck (14 slides)
Pass mark	80%
Price	USD 1,500

Learning objectives — on completion, learners will be able to:

- Document a complete, medically necessary clinical picture.
- Select a specific, covered ICD-10 code, using a provisional diagnosis when justified.
- Avoid defensive medicine and unjustified early repetition of tests.
- Classify claim rejections by cause and rework a rejected claim correctly.
- Distinguish the benefits added and amended by the new EBP and apply their limits, exclusions and pre-authorisation rules.

Course structure:

- Part A: Why it matters · Documentation strategy · ICD-10 coding · Defensive medicine & ordering · Rejections & coverage · "Fix the note"
- Part B: What changed · Benefits in practice · Reading rejections · Practice & close

Key interactive activities:

- Weak vs strong note side-by-side (same treatment, different claim outcome).
- Sort the full clinical picture; match symptoms to specific supported diagnoses.
- Sort ICD-10 codes the policy does not cover as a primary diagnosis.
- Joint-pain case scenario on initial work-up vs justified full panel.
- "Fix the note" case game with three realistic consultation notes.
- Sort added vs amended EBP benefits; match each benefit to its limit or condition.
- Classify an anonymised, rebuilt rejection log and rework a rejected claim.
- Adaptive "Covered or not?" practice with 15 patient requests.

Assessment and tracking:

17 scored activities with results by objective. The question bank can also be exported in QTI 2.1 format for reuse in other assessment tools.

Course-specific deliverables:

- 2 SCORM packages
- "Clear, strong, defensible" physician desk checklist (PDF)
- One-page EBP benefits summary (PDF)
- QTI 2.1 question-bank export

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Production volume:

≈36 screens · 17 scored activities · 4 scenarios, games and adaptive practice · ≈10 visuals and 2 job aids · ≈11 minutes of narration · 16 course-specific test cases.

Inputs needed from MY CLINIC:

- Regulator name (CCHI or CHI) and the policy effective date
- Repeat-test intervals and coverage rules to confirm
- Coding lead sign-off of every ICD-10 code; the official EBP document
- Anonymised rejection examples approved by the Data Protection Officer

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Course 5 — Clinical Governance: Quality, Patient Safety & Risk

Item	Details
Format	One SCORM package (~25 min)
Audience	All clinical staff (recommended for all staff)
Source	Quality, Safety and Risk Orientation video (8 min 56 s)
Pass mark	80%
Price	USD 1,500

Learning objectives — on completion, learners will be able to:

- Classify quality measures as structure, process or outcome, and order the FOCUS-PDCA cycle.
- Apply the six International Patient Safety Goals, including the two-identifier rule.
- Score a risk on the 5×5 matrix and choose a response (reduce, transfer, accept, avoid).
- Distinguish critical incidents, adverse events and near misses within a just, non-punitive culture.
- Report an incident through the correct channel within 24 hours.

Course structure:

- Welcome · The quality standard · Six patient safety goals · Risk management · Incidents · Reporting · Close

Key interactive activities:

- Anonymous “speak up” reflection to open the course.
- Sort quality measures into structure, process and outcome; order FOCUS-PDCA.
- Explore the six safety goals; “Which Mohammed?” patient-identification scenario.
- Match daily practices to the six goals; order the five moments of hand hygiene.
- Risk-matrix worked example, risk scoring and response matching.
- Iceberg classification of six events; end-of-shift near-miss reporting scenario.

Assessment and tracking:

11 scored activities with results by objective. The original video carries a third-party watermark and a machine-transcribed narration, so L2S will rebuild the content with new visuals and new narration based on a corrected transcript and MY CLINIC's policies.

Course-specific deliverables:

- 1 SCORM package
- New narration script based on the corrected transcript

Production volume:

≈22 screens · 11 scored activities · 2 scenarios · ≈8 diagrams and illustrations (quality model, six pillars, wristbands, risk matrix, iceberg) · ≈7 minutes of narration · 9 course-specific test cases.

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Inputs needed from MY CLINIC:

- IPSG policy wording, the corporate risk matrix and the incident-reporting policy
- Confirmation of the 24-hour rule and the falls-band colour
- Quality Director and Patient Safety Officer review
- Optional: a 60-second in-house “speak up” clip from the Quality Director

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Course 6 — IT: EHR Consultation Workflow

Item	Details
Format	One SCORM package (~25 min) of guided simulations
Audience	Physicians and clinical staff who document consultations in the EHR
Source	IT video (8 min 38 s, silent screen recording) — used only as a workflow reference
Pass mark	80%
Price	USD 1,500

Learning objectives — on completion, learners will be able to:

- Open a consultation and complete the mandatory fields.
- Add a primary diagnosis using ICD-10 code or name search.
- Place medication, lab and package orders linked to the assessed diagnosis.
- Produce the consultation summary, send education by SMS, and make an internal referral with follow-up.
- Put the complete nine-step consultation workflow in the correct order.

Course structure:

- The workflow · Open & document · Diagnosis · Orders · Summary & education · Referral & finish

Key interactive activities:

- Seven guided click-and-type simulations on a test patient: open the consultation, add the E11 diagnosis, medication order, lab package through the Form Wizard, consultation summary, SMS education, internal referral and follow-up.
- Find the mandatory fields on the consultation form.
- Questions on non-formulary items and why every order must link to a diagnosis (connects to the RCM course).
- Sort the full workflow into order.

Assessment and tracking:

11 scored activities with results by objective. Every step works with sound off: instructions are on screen and each screenshot has alt text.

Course-specific deliverables:

- 1 SCORM package
- EHR consultation quick-reference card (PDF)

Production volume:

≈20 screens · 11 scored activities · 7 guided simulations · ≈35 new screenshots captured on a test patient · ≈5 minutes of narration · 10 course-specific test cases.

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Inputs needed from MY CLINIC:

- A test environment, a test patient and a 2-hour screen-capture session with the EHR lead
- EHR version and confirmed button names
- Pharmacy-approved drug examples and the clinical disclaimer
- Data Protection Officer check of every screenshot

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Deliverables

- **Six interactive e-learning courses in English, delivered as eight SCORM packages** (HSE and RCM each have two parts), with a total seat time of about 4 hours.
- **For each course:**
  - Approved storyboard (screen-by-screen, Word/PDF)
  - Narration script, audio files, captions and transcript
  - Asset pack: diagrams, illustrations and screenshots with editable sources and a rights/provenance record
  - QA report with test results and the signed-off approval record
- **Job aids:** HR Quick Reference, physician documentation desk checklist, EBP benefits summary, EHR quick-reference card, bilingual emergency-code cards.
- **QTI 2.1 export** of the scored questions for reuse in other assessment tools.
- **LMS upload and testing:** L2S will upload the packages to **MY CLINIC**'s LMS (or hand them to the LMS administrator), test launch, scoring, completion and resume, and provide a short administrator guide for assigning courses and pulling reports.
- **MY CLINIC LMS administrator** will be able to:
  - Assign courses and set the learning time frame
  - Generate completion, score and objective-level reports
  - Monitor the progress of each trainee
- **Post-launch support:** 30 days of defect fixes after final delivery.

Scope of Work

- **Language:**
  - English (Arabic version available as an option, quoted separately)
- **Target audience:**
  - All new employees; RCM and EHR courses target physicians and clinical staff
- **Seat time:**
  - About 4 hours in total; 15 to 90 minutes per course; learners can pause and resume
- **Interactivity:**
  - High interactivity: branching scenarios, games, an escape room, guided software simulations, drag-and-drop, sorting, adaptive practice and knowledge checks with feedback
  - Responsive HTML5 (desktop, tablet and mobile browsers); no Flash
- **Narration:**
  - Professional English narration on narrated screens in two consistent voices (the guides Noor and Dr. Omar), with captions and transcripts
- **Standards:**
  - SCORM 1.2 or SCORM 2004 (4th edition), chosen to match MY CLINIC's LMS
  - Accessibility designed to WCAG 2.2 AA

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- **Branding:**
  - MY CLINIC brand colours, fonts and logo, in a clean minimalist style
- **Reviews:**
  - Up to two review rounds at each approval stage (objectives, storyboard, narration and media, built course)

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Project Phases

The following table summarises the phases L2S will go through and the points at which **L2S** and **MY CLINIC** will work together. All six courses move through the same phases; courses are staggered so that SME reviews are spread out.

Activities	L2S role	MY CLINIC role	Description	Duration
Phase 0 — Kick-off & Source Lock	Yes	Yes	Kick-off meeting; confirm LMS, SCORM version, brand guide and SME owners. Resolve the 80 course questions and 14 programme decisions. Collect photos, test accounts and policy confirmations.	Weeks 1–2
Phase 1 — Action Mapping & Objectives	Yes	Review	Define the business goal, on-the-job actions, learning objectives, assessment map and seat-time budget for each course.	Week 2
Phase 2 — Storyboard & Visual Design	Yes	Review & approve	Write screen-by-screen storyboards (~207 screens), scenario and game scripts with feedback, and question banks. Create the brand theme, visual style and the two guide characters, Noor and Dr. Omar. Automated storyboard checks.	Weeks 3–4
Phase 3 — Narration & Media Production	Yes	Approve voice & visuals	Write narration (~60 min, two voices) and the pronunciation guide; produce voice, captions and transcripts. Produce diagrams and illustrations, capture EHR and Oracle screens on test accounts, and design the job aids.	Weeks 4–6
Phase 4 — Development & Build	Yes	Preview review	Build the 8 SCORM packages; configure scoring, objectives, completion rules and tracking. Run automated course checks and share preview links.	Weeks 5–7
Phase 5 — QA, LMS Testing, Pilot & Delivery	Yes	Yes	Run 81 course-specific test cases plus the common checklist (accuracy, privacy, accessibility, scoring). Test in SCORM Cloud and MY CLINIC's LMS. Pilot with 5–8 new starters, fix issues, final sign-off and handover.	Weeks 7–9

Production Volume by Course

The table shows the amount of work planned in each course:

Course	Screens	Scored activities	Scenarios, games & simulations	Diagrams, images & screens	Narration (min)	Test cases
Health, Safety & Environment (HSE)	80	37	8	26	23	26
Welcome On Board: HR Essentials	36	15	4	11	12	12
Who Does What: Operations & My Care	13	4	1	6	4	8
RCM: Documentation, Coding & EBP Benefits	36	17	4	10	11	16

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Course	Screens	Scored activities	Scenarios, games & simulations	Diagrams, images & screens	Narration (min)	Test cases
Clinical Governance: Quality, Patient Safety & Risk	22	11	2	8	7	9
IT: EHR Consultation Workflow	20	11	7	35	5	10
Total	**207**	**95**	**26**	**96**	**62**	**81**

Roles and Responsibilities

Role	Responsibility
L2S Project Lead & Instructional Designer	Single point of contact; source analysis, action mapping, objectives, storyboards, scenario scripts, narration scripts; runs every review and the project plan
L2S E-Learning Developer	Builds the SCORM packages, interactions, scoring and tracking; LMS upload and testing
L2S Media Designer	Brand theme, diagrams, illustrations, screenshots, job aids and voice production
L2S QA	Test cases, accessibility checks, SCORM Cloud and LMS testing, pilot support
MY CLINIC Project Sponsor (Human Capital)	Approves scope, coordinates SMEs, signs off each course
MY CLINIC SMEs	Safety & Security Manager, Head of Human Capital, Director of Operations, RCM lead, Quality Director, EHR lead: answer Phase 0 questions and review content within 5 working days
MY CLINIC LMS Administrator & DPO	LMS access and settings; Data Protection Officer checks screenshots and anonymised data

Assumptions

The following assumptions were made when preparing this proposal:

- The courses are built from the eight source files in **MY CLINIC**'s induction folder. New topics (for example Clinical Risk or Infection Control, which appear on the HR agenda but have no source material) are not included.
- **MY CLINIC** is responsible for the accuracy of policy content; SMEs confirm the open questions in Phase 0 and review each stage within 5 working days.
- Up to two review rounds per approval stage. Changes after a stage is signed off are handled as a change request.
- Narration uses a professional English synthetic voice. A human voice actor, filming or the Arabic version are optional services.
- **MY CLINIC** provides its brand guide, photos where needed, an Oracle test account and an EHR test environment with a test patient.

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- **MY CLINIC**'s LMS supports SCORM 1.2 or SCORM 2004. LMS licences and hosting are not included.
- No real patient, staff or claim data will appear in any course.
- The price is a one-time development fee for use with **MY CLINIC** employees; there is no per-learner fee for courses hosted on **MY CLINIC**'s LMS.

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Investment

We declare this proposal binding and free of errors or omissions. Due diligence has been performed in order to ensure compliance with your requirements and address your practical needs.

LEAP TO SUCCESS FZ LLC offers to provide services in accordance with this Proposal and the cost for completion of this Project is as described in the below summary:

#	Description	Unit	Price USD	Total Price USD
1	Health, Safety & Environment (HSE) — design, development, narration, testing and LMS delivery	1 course	1,500	1,500
2	Welcome On Board: HR Essentials — design, development, narration, testing and LMS delivery	1 course	1,500	1,500
3	Who Does What: Operations & My Care — design, development, narration, testing and LMS delivery	1 course	1,500	1,500
4	RCM: Documentation, Coding & EBP Benefits — design, development, narration, testing and LMS delivery	1 course	1,500	1,500
5	Clinical Governance: Quality, Patient Safety & Risk — design, development, narration, testing and LMS delivery	1 course	1,500	1,500
6	IT: EHR Consultation Workflow — design, development, narration, testing and LMS delivery	1 course	1,500	1,500
	Total (6 courses)			**USD 9,000**

Optional services (not included; quoted on request):

- Arabic version of the courses
- Human voice talent for narration
- A narrated, captioned EHR walkthrough video
- Hosting on the L2S Learning Management System
- Annual content update package (policy, code and system changes)

Terms of Payment

The fees mentioned above are payable as follows:

- 50% upon proposal acceptance
- 30% upon approval of the storyboards (end of Phase 2)
- 20% upon final delivery of the SCORM packages
- *The stated amount is exclusive of VAT, Withholding Tax and other duties, taxes, and transfer-related charges which if applicable are payable by MY CLINIC in addition to the stated amount.*

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Proposal Validity

This proposal shall be valid for **15 days** from the date of receipt.

Deployment Start

Leap to Success FZ LLC will hold the kick-off meeting within one (1) week of proposal approval and expects to complete the project in about nine (9) to ten (10) weeks, provided **MY CLINIC**'s SME reviews are returned within five (5) working days.

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Approval

MY CLINIC reviewed the proposal Reference# **e2026-MC01** dated **14 - October - 2026** and found it in line with their requirements, and therefore below finds approval for the proposal.

In testament therewith, we affix the signature of our designated representative as a sign of our approval and acceptance of this proposal.

_____	_____
MY CLINIC	**DATE**
_____	_____
LEAP TO SUCCESS FZ LLC	**DATE**

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